

Policy

Off-Label Psychotropic Prescribing Guide for Children

Policy code	EMHS: 168
Original endorsement date	July 2016
Date of current endorsement	April 2025
Version	2.00
Functional subgroup	Mental Health
Summary	The Mental Health Act 2014 provides that a record must be made of off-label prescribing to a child (a person who is under 18 years of age) who is receiving involuntary mental health care. EMHS clinicians are required to obtain consent for the use of off-label medication from the parent or guardian when prescribing off-label treatments, irrespective of the child's status under the Act.
Policy owner	Executive Director Mental Health
Contact	Program Manager Mental Health Strategy
Review date	April 2028
NSQHS Standards (second edition)	Standard 1; Clinical Governance. Standard 2 Medication Safety
NSMHS Standard 2010	Standard 2 Safety; Standard 10; Delivery of Care
Impact Statement Declaration (ISD)	The Aboriginal consultation is reflected in this policy to ensure cultural appropriateness and positive health impacts on the Aboriginal community (ISD Record ID 3990)
Status	Active
Information Classification	OFFICIAL

1. Background

1.1. About this document

The Mental Health Act 2014 (the Act) provides that a record must be made of off-label prescribing to a child (a person who is under 18 years of age) who is receiving involuntary mental health care.

EMHS clinicians are required to obtain consent for the use of off-label medication from the child's parent or guardian when prescribing off-label treatments, regardless of the child's status under the Act.

Off-label treatment means the use of registered therapeutic goods other than in accordance with the approved Product Information for the registered therapeutic good. If the medication is being used outside of the age group, dose, indication or route as mandated by Therapeutic Goods Administration registration and/or official manufacturer's Product Information then this constitutes off-label prescribing.

This policy is to be used as a quick-reference guide to determine the licensed age and indication for psychotropic medication use in children.

Refer to:

- [TGA Product Information](#)
- [Mental Health Act 2014](#)
- [Office of the Chief Psychiatrist - Off-label prescription to a child who is an involuntary patient](#)
- [Department of Health S8 Stimulant Medicines](#)
- [Paediatric - WA Statewide Medicines Formulary](#)

And

- Appendix A - Flowchart for assessing appropriateness of off-label medicine use.

2. Responsibilities of Clinician

As an EMHS clinician you have a professional, ethical and legal obligation to:

- evaluate the evidence for yourself.
- inform the child and their parent/guardian that the medication is off-label.
- discuss alternatives.
- provide the child and their parent/guardian with consumer product information, to provide an opportunity to study this information and to answer any questions.
- obtain consent and document in the child's healthcare record.

Metabolic screening must be undertaken whenever prescribing psychotropic medication.

3. Notification Requirement

EMHS clinicians are required to comply with the recording and reporting requirements set out in section 304 of the Act, providing notification to the Office of the Chief Psychiatrist (OCP).

- Use the MHA2014 s.304 Off-label treatment provided to a child who is an involuntary patient – Notification Form
- This form can be accessed via the OCP website [Off-label prescription to a child who is an involuntary patient | Chief Psychiatrist](#) (password: OCP2003)

To assist clinicians to make informed decisions about prescribing off label, and to create a platform to obtain informed consent, resources have been developed by Child and Adolescent Mental Health Service and are available in the [CAMHS Medication Safety Toolbox](#).

4. Consent and Information

Children, parents, and guardians must be provided with sufficient verbal and written information regarding the treatment to enable informed decision making. Parents and guardians must be provided with information to facilitate monitoring and identification of side effects.

In accordance with best practice, and as per OCP and Department of Health recommendations, clinicians must record parental consent for off-label treatment in the child's healthcare record.

The [Choice and Medication](#) website may be used as a resource to provide children, parents, and guardians with information on psychotropic medications. Many of the medication leaflets refer to common off-label indications.

Willingness to continue medication may be influenced by personal experience, values and response to treatment. Children, parents, and guardians must be informed that they have the right to withdraw their consent at any time by notifying the clinician.

5. Prescribing Greater than Maximum Dose

Prescribing greater than maximum dose is a type of off-label prescribing that refers to the prescription of a single medication that exceeds the licensed maximum dose.

It may also refer to the prescription of two (or more) medications from the same class (e.g. antipsychotics) such that when the individual doses are expressed as a percentage of their maximum licensed dose, the sum of the percentages is greater than 100%. This is considered 'off-label' prescribing for the purposes of this policy even if one or all medications assessed individually do not exceed the licensed dose.

A documented plan for close physical monitoring and identification of adverse effects is essential as many side effects are dose related therefore prescription of high or combined doses are likely to result in an increased side effect burden.

6. Class of Medications

All information contained in this guideline is subject to change. Refer to the product information for more detailed information as required. This guideline is not intended to be used as a primary reference for medication dosages.

Despite off-label status of a psychotropic in Australia, there may be reasonable evidence in favour of its use. The United States Food and Drug Administration (FDA) approval status is also displayed to guide clinicians in decision making.

Refer to Appendix B for abbreviations used in this guide.

Antidepressants

	Australian Licensed Use for <18 years in psychiatry	Statewide Medicines Formulary	FDA licensed Use for <18 years in psychiatry
Citalopram	No	Second line treatment MD in ≥ 8 years Management anxiety ≥ 8 years	No
Escitalopram	No	Second line treatment MD in ≥ 8 years Management anxiety ≥ 8 years	MD >12 years
Fluoxetine	No	First line for major depression in patients ≥ 8 years. For anxiety in patients ≥ 8 years. For autism spectrum disorders; (repetitive behaviour). For bulimia nervosa	MD >8 years OCD >7 years Depressed BPAD1 in combo with olanzapine
Fluvoxamine	>8 years in OCD	For prescription under the direction of a Psychiatrist	OCD >8 years
Sertraline	>6 years in OCD	First line for major depression in patients ≥ 8 years. For anxiety in patients ≥ 8 years. For autism spectrum disorders;	OCD >6 years

		(repetitive behaviour).	
Duloxetine	No	Unrestricted	GAD >7 years
Desvenlafaxine	No	For prescription under the direction of a Psychiatrist and for use as per PBS indications and criteria	No
Venlafaxine	No	For prescription under the direction of a Psychiatrist	No
Amitriptyline	No	Unrestricted	Depression >12 years
Clomipramine	No	Unrestricted	OCD >10 years
Nortriptyline	No	Unrestricted	Depression
Mirtazapine	No	Unrestricted	No
Vortioxetine	No	Not listed	No
Agomelatine	No	Not Listed	No

Oral Antipsychotics

	Australian Licensed Use for under 18's in psychiatry	Statewide Medicines Formulary	FDA licensed Use for under 18's in psychiatry
Aripiprazole	No	For prescription by a Paediatrician or Psychiatrist to treat children over 6 years of age with severe aggression/irritability combined with an intellectual disability and/or an autism spectrum disorder when other psycho-social interventions have failed	SCZ >13 years Tourette's >6 years BPAD1 monotherapy >10 years BPAD1 combination with lithium or valproate >10 years AUT >6 years

Olanzapine	No	For use under the direction of a psychiatrist, neurologist or paediatrician For use as per the CAHS Acute Agitation and Arousal Guideline in children >6 years	SCZ >13 years BPAD1 with depression >10 years in combination with fluoxetine BPAD1, acute mixed or manic episode >13 years
Quetiapine	BPAD1 >10 years SCZ >13 years	For use under the direction of a psychiatrist, neurologist or paediatrician	SCZ >13 years BPAD1 monotherapy >10 years
Risperidone	SCZ >15 years BD >5 years AUT 0+	For use under the direction of a psychiatrist, neurologist or paediatrician	SCZ >13 years AUT 5-16 years BPAD1 >10 years
Clozapine	Treatment resistant SCZ >16 years	Not listed	No
Chlorpromazine	BD >5 years	For use under the direction of a psychiatrist	Severe Problem behaviour >12 years
Zuclopenthixol acetate	No	For use under the direction of a psychiatrist	No
Paliperidone	No	For use under the direction of a psychiatrist neurologist or paediatrician For use as per local hospital's Acute Agitation and Arousal Guideline in Children > 6 years	SCZ >12 years
Lurasidone	SCZ > 13 years	For use under the direction of a psychiatrist, neurologist or paediatrician	BPAD1 monotherapy > 10 years SCZ >13 years
Brexpiprazole	No	Not listed	SCZ >13 years
Ziprasidone	No	Not listed	No

Haloperidol	Tourettes >3 years	Unrestricted	Tourettes >3 years SCZ >3 years Hyperactive behaviour >3 years Severe problematic behaviour >3 years
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Parenteral Antipsychotics

	Australian Licensed Use for under 18's in psychiatry	Statewide Medicines Formulary	FDA licensed Use for under 18's in psychiatry
Olanzapine	No	For use under the direction of a psychiatrist, neurologist or paediatrician For use as per the CAHS Acute Agitation and Arousal Guideline in children >6 years	No
Zuclopenthixol acetate	No	For use under the direction of a psychiatrist	No

Depot Antipsychotics

	Australian Licensed Use for under 18's in psychiatry	Statewide Medicines Formulary	FDA licensed Use for under 18's in psychiatry
Aripiprazole	No	For prescription by a Paediatrician or Psychiatrist to treat children over 6 years of age with severe aggression/irritability combined with an intellectual disability and/or an autism spectrum disorder when other psycho-social interventions have failed	No
Olanzapine	No	Not listed	No
Risperidone	No	Not listed	No

Paliperidone	No	For use as per PBS indications and criteria under the direction of a psychiatrist	No
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Mood Stabilisers

	Australian Licensed Use for under 18's in psychiatry	Statewide Medicines Formulary	FDA licensed Use for under 18's in psychiatry
Lamotrigine	No	For borderline personality disorder under the direction of a psychiatrist. For bipolar depression under the direction of a psychiatrist For use as per PBS indications and criteria	No
Lithium	>12 years	To be prescribed under the direction of a Child and Adolescent Psychiatrist	Immediate release: BPAD1 maintenance & acute >7 years Controlled release: BPAD1 maintenance & acute >12 years
Carbamazepine	No	Unrestricted	No
Sodium Valproate	No	Unrestricted	No

Stimulants

	Australian Licensed Use for under 18's in psychiatry	Statewide Medicines Formulary	FDA licensed Use for under 18's in psychiatry
Methylphenidate	ADHD > 6 years	For use as per PBS indications and criteria	ADHD >6 years
Dexamphetamine	ADHD >3 years	For use as per PBS indications and criteria	ADHD 3-16 years

Lisdexamphetamine	ADHD > 6 years	For use as per PBS indications and criteria	ADHD >6 years
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Benzodiazepines

	Australian Licensed Use for under 18's in psychiatry	Statewide Medicines Formulary	FDA licensed Use for under 18's in psychiatry
Midazolam	No	Unrestricted	No
Clonazepam (tablets, drops & IV)	No	Unrestricted	No
Lorazepam oral	Anxiety >16 years	Unrestricted	Anxiety >12 years
Lorazepam IM (SAS)	No	Can only be used where local hospital guidelines exist for the management of acute agitation and arousal management for older adolescent mental health inpatients aged 16 to 17 years	Anxiety >12 years
Temazepam	>16 years	Unrestricted	No
Diazepam oral	Yes	Unrestricted	Anxiety >6 months

Other

	Australian Licensed Use for under 18's in psychiatry	Statewide Medicines Formulary	FDA licensed Use for under 18's in psychiatry
Nicotine Replacement patch	>12 years	Unrestricted	No
Clonidine oral	No	Unrestricted	ADHD >6 years
Atomoxetine	ADHD >6 years	For use as per PBS indications and criteria	ADHD >6 years

Guanfacine	ADHD >6 years	For use as per PBS indications and criteria	ADHD >6 years
Melatonin	No	Not listed	No
Zolpidem	No	For prescription by a Paediatrician for the treatment of adolescents with rebound insomnia.	No

7. Compliance monitoring and legislative penalties

Compliance monitoring

The EMHS Executive Director Mental Health is responsible for ensuring EMHS mental health staff are made aware of and comply with this policy, and that compliance risks and training requirements are managed.

Staff are to be reminded that compliance with all policies is mandatory.

Legislative obligations

There are legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

8. Legislation

- Mental Health Act 2014. (Reporting and Recording requirement Section 304)

9. Additional references

- [TGA Product Information](#)
- [World Health Organisation - Promoting Safety of Medicines for Children](#)
- [Choice Medication - Consumer and Carer Medication Information](#)
- [Chief Psychiatrist Standards for Clinical Care](#)
- [Department of Health S8 Stimulant Medicines](#)
- [Council of Australian Therapeutic Advisory Groups \(CATAG\) - Rethinking medicines decision-making in Australian Hospitals](#)

6.1 WA Health policies

- [MP 0175/22 Consent to Treatment Policy](#)

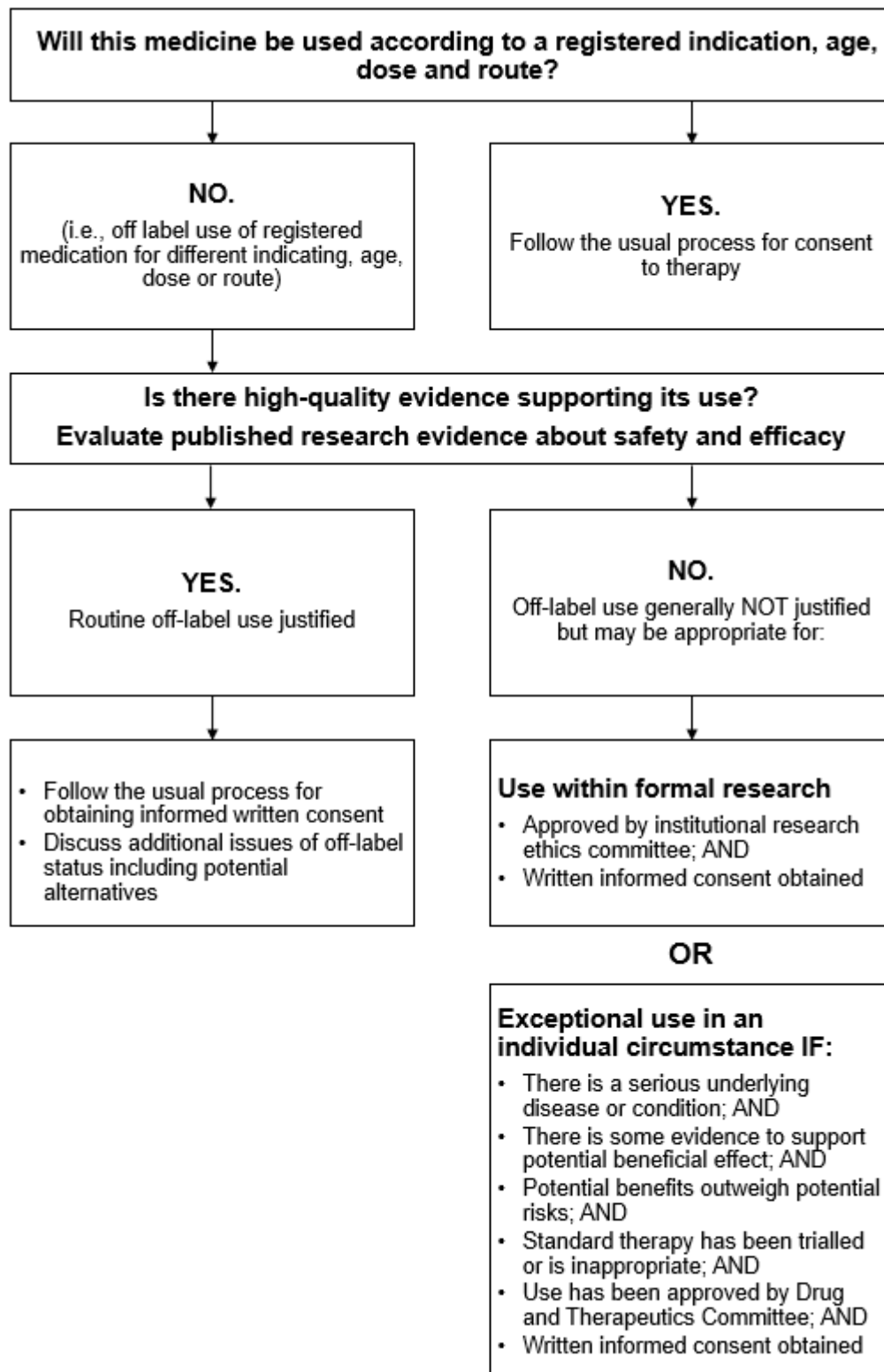
6.2 EMHS policies

- [EMHS Consent Policy](#)

Appendix A – Flowchart

These guidelines should be used in conjunction with the flowchart below.

Off-label Psychotropic Prescribing Guide for Children flowchart



Appendix B – Abbreviations used in Guide

BPAD1	Bipolar Affective Disorder 1
SCZ	Schizophrenia
BD	Behavioural Disturbance in Intellectual Disability
AUT	Autism
ADHD	Attention Deficit Hyperactivity Disorder
MD	Major Depression
OCD	Obsessive Compulsive Disorder
GAD	Generalised anxiety disorder